



ISSUED, VALID FROM 14 SEPTEMBER 2026

Period Delay (Norethisterone)

Patient Group Direction, version 010, issued 14 September 2026. Norethisterone 5mg tablets, women aged 16 years and over.

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The three numbers that changed

- **MAXIMUM 14 DAYS** of treatment. 14 days is the figure that now applies throughout.
- **MAXIMUM 42 TABLETS** per supply. At 5mg three times daily that is exactly 14 days.
- **VTE risk factors** are now **EXCLUSIONS**, not cautions. See the gate below.

Why the venous thromboembolism gate is real

Norethisterone at 5mg three times daily is a high dose. A clinically significant proportion is metabolised to ethinylestradiol, so the venous thromboembolism risk at this dose sits closer to that of a combined oral contraceptive than to a progestogen-only pill. It is not a low-risk hormone taken for a fortnight.

- **Long-haul travel is both the commonest reason a woman asks for this and one of the risk factors. That tension is resolved in favour of safety: a journey of 4 hours or more during the course, or within 2 weeks of finishing it, is an exclusion.**
- **Where a woman is excluded only because of travel, tell her what she can do instead. See Appendix 2.**

Patients aged 16 and 17

- **This PGD covers 16 and 17 year olds. Every supply to a patient under 18 requires a recorded competence assessment and a recorded consideration of safeguarding.**
- **Ask why the delay is wanted and who has suggested it. Where the request appears to come from someone other than the patient, or the reason given is inconsistent, do not supply. Refer.**
- **Consider whether the request may indicate coercion, exploitation, or an attempt to conceal something. A request to delay a period is not neutral in a young person.**
- **Record the assessment, not just its conclusion.**



Norethisterone 5mg for the short term delay of menstruation



Summary of UKMEC 2025 guidance and the Primolut N SmPC for norethisterone period delay

College of Sexual and Reproductive Healthcare (CoSRH, formerly FSRH), UK Medical Eligibility Criteria for Contraceptive Use, UKMEC 2025: Summary Tables and Summary of key changes, published December 2025 (cosrh.org). Bayer plc, Primolut N 5 mg tablets, Summary of Product Characteristics, text revised 13 December 2018, last updated on the emc 11 January 2019 (medicines.org.uk). Summarised 10 September 2026.

Why the UKMEC combined hormonal contraception column is applied by analogy

The UKMEC does not cover norethisterone used for period delay. Its own introduction states that it relates to safety and not efficacy, that its recommendations are for contraceptive purposes only, and that where a method is used for a non-contraceptive indication the risk-benefit profile and eligibility criteria may differ.

The Primolut N SmPC states that after oral administration norethisterone is partly metabolised to ethinylestradiol, resulting in an equivalent dose of about 4 to 6 micrograms of ethinylestradiol per 1 mg of orally administered norethisterone or norethisterone acetate.

Because of this partial conversion, the SmPC states that administration of Primolut N is expected to result in similar pharmacological effects to those seen with combined oral contraceptives, and that the general warnings associated with COC use should therefore also be considered.

The SmPC lists the COC-type risk factors for thromboembolic events that should be considered: age; obesity (BMI over 30 kg/m²); a positive family history of venous or arterial thromboembolism in a sibling or parent at a relatively early age (refer for specialist advice if a hereditary predisposition is known or suspected); prolonged immobilisation, major surgery, any surgery to the legs or major trauma; smoking (the risk increases further with heavier smoking and increasing age, especially over 35 years); dyslipoproteinaemia; hypertension; migraine (an increase in frequency or severity during use may be prodromal of a cerebrovascular event and is a reason for immediate discontinuation); valvular heart disease; and atrial fibrillation.

The SmPC also notes that diabetes mellitus, SLE, haemolytic uraemic syndrome, chronic inflammatory bowel disease and sickle cell disease have been associated with adverse circulatory events, and that when weighing risk and benefit the risk of thrombosis associated with pregnancy is higher than that associated with COC use.

The UKMEC CHC column is therefore used as the structured way of grading those same risk factors for this indication, on the SmPC's own statement that the product behaves like a COC.

What UKMEC categories 1 to 4 mean

Category 1: a condition for which there is no restriction for the use of the method.

Category 2: a condition where the advantages of using the method generally outweigh the theoretical or proven risks.

Category 3: a condition where the theoretical or proven risks usually outweigh the advantages. Provision requires expert clinical judgement and/or referral to a specialist provider, since use is not usually recommended unless other more appropriate methods are not available or not acceptable.

Category 4: a condition which represents an unacceptable health risk if the method is used.

Multiple category 2 conditions relating to the same risk may indicate a cumulative risk, and clinical judgement must be used to decide whether risks outweigh benefits. Multiple category 3 conditions may pose an unacceptable health risk. Multiple risk factors are defined as more than one risk factor, and where more than one is present clinical judgement must be applied.

Absence of a condition from the UKMEC does not always mean the method is safe.

Examples of VTE risk factors given in UKMEC 2025 include previous VTE, cancer, recent major surgery, recent trauma, significant immobility, high BMI, pregnancy and the postnatal period, inflammatory disorders, antiphospholipid antibody syndrome and other thrombotic disorders. A family history of unprovoked VTE is a stronger risk factor than a family history of provoked VTE.

The UKMEC 2025 combined hormonal contraception categories for this PGD

Age: menarche to under 40 years is category 1; 40 to 50 years is category 2.

Smoking under 35 years is category 2. Smoking at age 35 or over: fewer than 15 cigarettes a day is category 3; 15 or more cigarettes a day is category 4; stopped smoking less than 1 year ago is category 3; stopped smoking 1 year or more ago is category 2.

E-cigarettes: UKMEC does not include e-cigarettes because there is insufficient evidence to establish the risks, but given the unknown long-term cardiovascular risks, alternatives to CHC should be prioritised.

Obesity: BMI 30 to 34.9 kg/m² is category 2; BMI 35 kg/m² or over is category 3. The same categories apply after bariatric surgery according to current BMI.

Venous thromboembolism: a history of VTE, or current VTE on anticoagulants, is category 4. A family history of VTE in a first-degree relative is category 3. Major surgery is category 4. Immobility (for example wheelchair use or chronic conditions) is category 3. Multiple risk factors for VTE (additional examples include cancer, high BMI, thrombotic or inflammatory disorders) is category 4. Known thrombogenic mutations (for example factor V Leiden, prothrombin mutation, protein S, protein C and antithrombin deficiencies) is category 4. Superficial venous thrombosis is category 2 and varicose veins category 1.

Migraine with aura at any age is category 4. A history of migraine with aura 5 or more years ago, at any age, is category 3.

Migraine without aura at any age is category 2 for initiation and 3 for continuation. Non-migrainous headache is category 1 for initiation and 2 for continuation.



Hypertension: controlled hypertension is category 3. Consistently elevated, properly taken blood pressure at stage 1 (clinic systolic 140 to 159 and/or diastolic 90 to 99, or home 135 to 149 and/or 85 to 94) is category 3. Stage 2 or 3 hypertension (clinic systolic 160 or more and/or diastolic 100 or more, or home 150 or more and/or 95 or more) is category 4. Hypertension with vascular disease is category 4. A history of high blood pressure during pregnancy is category 2. Multiple risk factors for cardiovascular disease (for example smoking, diabetes, hypertension, obesity, dyslipidaemia) is category 3. Also relevant to the SmPC contraindications: breast cancer currently being treated is category 4 and completed treatment is category 3; carriers of high-risk gene mutations such as BRCA1 or BRCA2 are category 3; severe decompensated cirrhosis, hepatocellular adenoma and hepatocellular carcinoma are category 4; positive antiphospholipid antibodies in SLE is category 4; stroke or TIA, current or past ischaemic heart disease, atrial fibrillation and complicated valvular heart disease are category 4.

Drug interactions are not graded in the UKMEC and should be considered separately using CoSRH guidance, the BNF, the SmPC or Stockley's.

The Primolut N SmPC: indication, dose and timing for period delay

Each tablet contains norethisterone 5 mg (with lactose). The licensed indications are metrorrhagia haemorrhagica, premenstrual syndrome, postponement of menstruation, endometriosis, menorrhagia and dysmenorrhoea. It is not intended for use in children.

Postponement of menstruation: the SmPC states this is possible in cases of too frequent menstrual bleeding and in special circumstances, for example operations, travel or sports.

Dose: 1 tablet (5 mg) three times daily, starting 3 days before the expected onset of menstruation and continuing for not longer than 10 to 14 days.

A normal period should occur 2 to 3 days after the patient has stopped taking the tablets.

The SmPC states that this method should be restricted to users who are not at risk of pregnancy during the treatment cycle.

Tablets are swallowed whole with some liquid.

If menstrual bleeding fails to follow a course, or if the patient wishes to postpone menstruation again in special circumstances, the possibility of pregnancy must be excluded before a further course is given.

Contraindications in the SmPC

Hypersensitivity to norethisterone or any excipient.

Known or suspected pregnancy, and lactation (Primolut N can pass into breast milk and should be avoided during lactation).

Previous idiopathic or current venous thromboembolism (deep vein thrombosis, pulmonary embolism).

Active or recent arterial thromboembolic disease (for example angina, myocardial infarction), and the presence or a history of prodromi of a thrombosis (for example transient ischaemic attack, angina pectoris).

A high risk of venous or arterial thrombosis.

History of migraine with focal neurological symptoms.

Diabetes mellitus with vascular involvement.

Presence or history of severe hepatic disease as long as liver function values have not returned to normal, and previous or existing liver tumours, benign or malignant.

Known, past or suspected sex hormone-dependent malignancies, including of the genital organs or breast cancer.

History during pregnancy of idiopathic jaundice or severe pruritus.

Undiagnosed genital bleeding, and untreated endometrial hyperplasia.

Concomitant use with medicinal products containing ombitasvir/paritaprevir/ritonavir and dasabuvir, because of ALT elevations seen with ethinylestradiol-containing products; Primolut N can be restarted 2 weeks after that regimen is completed.

Porphyria is not listed as a contraindication in the SmPC. It appears in the SmPC among conditions reported to occur or deteriorate with both pregnancy and COC use, where the evidence of an association is described as inconclusive, alongside cholestatic jaundice or pruritus, gallstones, SLE, haemolytic uraemic syndrome, Sydenham's chorea, herpes gestationis and otosclerosis-related hearing loss.

Patients with rare hereditary galactose intolerance, Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Warnings, precautions, interactions and adverse effects in the SmPC

A complete personal and family medical history should be taken, with examination guided by the contraindications and warnings, including measurement of blood pressure.

Therapy should be discontinued immediately if any of the following occur: new onset of migraine-type headaches or more frequent unusually severe headaches; sudden perceptual disorders such as disturbances of vision or hearing; first signs of thrombophlebitis or thromboembolic symptoms, or pain and tightness in the chest; pending operations (six weeks beforehand) or immobilisation; onset of jaundice, deterioration in liver function, anicteric hepatitis or general pruritus; significant increase in blood pressure; pregnancy.

Patients with a history of VTE or known thrombophilic states have an increased risk of VTE and steroid hormone treatment may add to it. A personal or strong family history of thromboembolism or recurrent spontaneous abortion should be investigated, and until thrombophilic factors have been evaluated or anticoagulation started, use of progestogens should be viewed as contraindicated. Where a patient is already anticoagulated, risks and benefits should be carefully considered.



Where prolonged immobilisation is likely after elective surgery, particularly abdominal or lower limb orthopaedic surgery, consider stopping progestogen therapy 4 to 6 weeks pre-operatively and not restarting until fully remobilised. In the COC-derived warnings the interval is at least four weeks before elective surgery and two weeks after complete remobilisation. Norethisterone can influence carbohydrate metabolism, so parameters should be examined carefully in diabetics before and during treatment. Progestogens may cause fluid retention, so special care is needed in epilepsy, migraine, asthma, cardiac dysfunction and renal dysfunction. Patients with a history of depression should be carefully observed and the drug stopped if depression recurs to a serious degree.

Chloasma may occur, especially with a history of chloasma gravidarum; women with a tendency to chloasma should minimise sun or ultraviolet exposure. Anyone developing acute impairment of vision, proptosis, diplopia or migraine headache should be evaluated ophthalmologically before continuing. In rare cases benign, and more rarely malignant, liver tumours have been reported in users of hormonal substances, occasionally with life-threatening intra-abdominal haemorrhage. The SmPC reproduces the COC epidemiology: VTE in users of low-oestrogen oral contraceptives is about 20 to 40 cases per 100,000 women-years compared with 5 to 10 in non-users and about 60 per 100,000 pregnancies; excess risk is highest in the first year of use or on restarting after a break of at least a month; VTE may be fatal in 1 to 2 percent of cases. Common signs of VTE are severe pain in the calf of one leg, swelling of the lower leg, sudden breathlessness and chest pain. Signs of arterial thromboembolism include sudden severe chest pain, sudden unexplained cough, and any unusual severe prolonged headache, especially if first-ever or worsening or with sudden loss of vision or diplopia, aphasia, vertigo, collapse, or sudden weakness or marked numbness on one side. A slightly increased relative risk of breast cancer diagnosis (RR 1.24) in current COC users is described, disappearing over 10 years after stopping.

Interactions: enzyme inducers such as phenytoin, barbiturates, bosentan, primidone, carbamazepine, rifampicin, ritonavir, nevirapine, efavirenz, and possibly oxcarbazepine, topiramate, felbamate, griseofulvin and St John's wort increase clearance of sex hormones and may change the bleeding profile or reduce the therapeutic effect; induction can persist for about 4 weeks after the inducer is stopped. Strong and moderate CYP3A4 inhibitors (azole antifungals, verapamil, macrolides, diltiazem, grapefruit juice) can raise concentrations. Progestogens may increase ciclosporin and decrease lamotrigine levels.

Progestogens may affect laboratory tests of hepatic function, thyroid function and coagulation.

Adverse effects are more common in the first months and subside with continued use. Very common: uterine or vaginal bleeding including spotting, and hypomenorrhoea (frequencies from the endometriosis indication). Common: headache, nausea, amenorrhoea, oedema. Uncommon: migraine. Rare: hypersensitivity reactions, urticaria, rash. Very rare: visual disturbances, dyspnoea. Frequency not known: dizziness, depression aggravated, abdominal pain, cholestasis, jaundice. No ill effects from overdose have been reported.

Pregnancy: contraindicated; the SmPC states the product may provoke signs of virilisation in female fetuses if given from day 45 of pregnancy onwards. No effect on ability to drive is known.

What to tell the patient

How to take it: one 5 mg tablet three times a day, swallowed whole with liquid, starting 3 days before the period is expected, and for no longer than 10 to 14 days in total.

A normal period should arrive 2 to 3 days after the last tablet.

It is not a contraceptive. The SmPC restricts this use to women who are not at risk of pregnancy during the treatment cycle, and if no bleed follows the course, pregnancy must be excluded before any further course.

Stop the tablets and seek medical help at once for severe pain or swelling in one calf, sudden breathlessness or chest pain, a new migraine-type or unusually severe headache, sudden changes in vision or hearing, sudden weakness or numbness on one side, jaundice or generalised itching.

Tell the pharmacist or prescriber about any planned operation or period of immobility, since treatment should be stopped six weeks before a pending operation.

Expected effects include spotting or breakthrough bleeding, headache, nausea, fluid retention and breast or mood changes; effects are more common in the first months of use. Women prone to chloasma should minimise sun exposure.

Report any suspected side effect through the MHRA Yellow Card scheme.

Patient Group Direction for the supply of norethisterone 5mg tablets to delay menstruation in women aged 16 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for norethisterone 5mg tablets, the BNF and NICE CKS.
- All users must understand that norethisterone at this dose carries a venous thromboembolism risk closer to a combined oral contraceptive than to a progestogen-only pill, and must be able to explain that to a patient.
- All users must be competent to assess competence and consent in a 16 or 17 year old, and to recognise and act on safeguarding concerns including coercion and exploitation.
- All users must know the local safeguarding referral route and be able to use it without delay.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to this medicine.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Short term delay of menstruation in a woman aged 16 years and over with a regular menstrual cycle, who is not using hormonal contraception, and who wishes to postpone her period for a personal, social, religious or travel-related reason. This is a lifestyle supply, not a treatment for a medical condition.
Inclusion criteria	• Female, aged 16 years or over. • A regular, predictable menstrual cycle, so that the start date can be calculated. • The expected date of the next period is known and is more than 3 days away. • Not using hormonal contraception. Women on a monophasic combined pill should run packs back to back instead, which is safer and free. See Appendix 2. • Pregnancy excluded. See the pregnancy row below. • Valid informed consent obtained. Where the patient is 16 or 17, a competence and safeguarding assessment recorded. • No exclusion criterion present.
Exclusion criteria: venous thromboembolism gate, mirroring UKMEC 2025	EXCLUDE and refer where ANY of the following is present. The list is built from the UKMEC 2025 category 3 and 4 conditions for combined hormonal contraception, applied by analogy as explained in the guidance summary, together with the Primolut N SPC contraindications and further cardiovascular and oncological conditions; it is deliberately wider than UKMEC 3 and 4, not a mirror of them. • Personal history of venous thromboembolism, deep vein thrombosis, pulmonary embolism, stroke, transient ischaemic attack, myocardial infarction or any arterial disease. • Known thrombophilia, or a first degree relative with venous thromboembolism under the age of 45. • CURRENTLY SMOKES, any amount, any age. UKMEC grades smoking under 35 as category 2 and at 35 or over as category 3 or 4; this PGD excludes all current smokers, because this is a lifestyle supply and the risk is avoidable. • STOPPED SMOKING LESS THAN A YEAR AGO, any age. Ask this separately: a question that only asks "do you smoke" misclassifies a recent quitter.

	• BMI 30 kg/m² OR ABOVE. UKMEC grades 30 to 34.9 as category 2 and 35 or over as category 3; this PGD excludes from 30. Measure or ask for height and weight; do not estimate by looking. • Surgery under general anaesthetic within the last 6 weeks, or planned during or within 2 weeks of the course. • Any current or expected period of immobility, including a leg in plaster or being bed-bound. • Active cancer, or cancer treated within the last 12 months. • A FLIGHT, COACH, TRAIN OR CAR JOURNEY OF 4 HOURS OR MORE during the course, or within 2 weeks of finishing it. This will exclude many holiday requests; that is intended. Give the alternatives in Appendix 2.
Exclusion criteria: all other	EXCLUDE, refer, where any of the following applies. • Known or suspected pregnancy, or pregnancy cannot be excluded. • Breastfeeding. • Aged under 16. Male. • Irregular or unpredictable menstrual cycle. • Undiagnosed vaginal bleeding, bleeding between periods, or bleeding after sex. Refer; this needs a diagnosis, not a delay. • Liver dysfunction, active liver disease, a history of jaundice in pregnancy, or a liver tumour. • Any hormone sensitive cancer, including breast cancer, current or past. • Migraine with aura, or any migraine with focal neurological symptoms, current or past. • Acute porphyria, or severe pruritus in a previous pregnancy. • Hypersensitivity to norethisterone or to any excipient in the product supplied. • Taking an enzyme inducing medicine: rifampicin, rifabutin, carbamazepine, phenytoin, phenobarbital, primidone, topiramate, efavirenz, ritonavir or St John's wort. These reduce norethisterone efficacy and the delay is likely to fail. • Taking ciclosporin, or lamotrigine as monotherapy. • Diabetes with vascular complications. • Hypertension of any grade, treated or untreated, a blood pressure of 140/90 or above measured today, or a history of hypertension in pregnancy. • Atrial fibrillation, or valvular or congenital heart disease. • Systemic lupus erythematosus, or antiphospholipid antibodies or antiphospholipid syndrome. • Known BRCA1 or BRCA2 carrier status, or current or past breast cancer. • Dyslipidaemia together with any other cardiovascular risk factor (smoking is already excluded; this covers obesity below the BMI threshold, diabetes, hypertension and family history). • Aged 16 or 17 and the competence or safeguarding assessment is not satisfied, or the pharmacist has any concern about coercion. • Already supplied norethisterone for period delay twice in the last 6 months. Refer; repeated requests need review, and total treatment must not exceed 30 days in any 6 month period. • Valid informed consent not obtained.

Excluding pregnancy	Do not supply until pregnancy has been excluded. Ask, in these terms: • "When did your last period start, and was it normal for you?" • "Have you had sex without contraception, or had a contraceptive failure, since that period?" Where the last period was normal, on time, and there has been no unprotected sex since, pregnancy can be excluded on history. Otherwise, do not supply until a pregnancy test taken no earlier than 21 days after the last unprotected sex is negative. Record the date and result. Tell every patient that if her period does not arrive within a few days of finishing the course, she must do a pregnancy test.
Cautions	Norethisterone is NOT a contraceptive. A woman relying on it will become pregnant. Advise condoms or another method throughout and afterwards. May affect blood glucose. Advise a woman with diabetes to monitor more closely during the course. May cause mood changes, breast tenderness, nausea, headache and breakthrough bleeding. May cause fluid retention. Take care where a condition could be aggravated by it, including epilepsy, migraine, asthma, and cardiac or renal impairment. Stop the medicine and seek medical advice at once for: jaundice, a significant rise in blood pressure, a new migraine-type headache, sudden visual or hearing disturbance, any possible clot symptom, or suspected pregnancy. DEPRESSION AND MOOD. UKMEC 2025 REMOVED depression as a categorised condition and replaced it with a statement: there is not consistent evidence that hormonal contraceptives worsen or improve anxiety or mood disorders in those with pre-existing conditions. A history of depression is therefore NOT an exclusion from this PGD. What is required instead is individualised counselling: tell every woman that mood change is a recognised effect, ask her to monitor her mood, and tell her to seek follow up if it deteriorates. Record that you did. Where a woman has a history of depression, the Primolut N SPC directs that she be carefully observed and the medicine discontinued if depression recurs to a serious degree. Say that to her in plain terms: if your mood drops noticeably, stop taking it and speak to us or your GP. EXCLUDE and refer only where there is current severe depression, active suicidal ideation, or any safeguarding concern. That is a clinical judgement, not a tick box, and the reason must be recorded.
Actions if the patient is excluded or declines	Explain why supply is not appropriate, and give the alternatives in Appendix 2 rather than leaving her with nothing. Where the exclusion is travel-related, say so plainly and explain the reason. Where a safeguarding concern exists, follow the local safeguarding route the same day and record what was done. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.

The medicine

Name, form and strength	Norethisterone 5mg tablets.
Legal category	POM.
Dose and frequency	One 5mg tablet three times daily, starting 3 days before the expected onset of menstruation. All three doses are taken every day of the course.

Quantity to be supplied	MAXIMUM 42 TABLETS. That is 14 days at three tablets daily. Supply only the number of days actually needed, up to that maximum. Do not round up to a pack.
Maximum treatment period	14 DAYS. No extension under this PGD. Total norethisterone for period delay must not exceed 30 days in any 6 month period, and no patient may be supplied more than twice in 6 months under this PGD.
Route and method	Oral.
Storage	Store below 25 degrees Celsius in a dry place, away from direct sunlight.
Drug interactions	Enzyme inducing medicines reduce efficacy and are handled as exclusions rather than cautions: rifampicin, rifabutin, carbamazepine, phenytoin, phenobarbital, primidone, topiramate, efavirenz, ritonavir and St John's wort. Also excluded: ciclosporin, and lamotrigine used as monotherapy. Check the current SPC and BNF against the patient's full medicine list, including anything bought over the counter or online.
Adverse effects	Common: nausea, headache, breast tenderness, mood changes, breakthrough bleeding, fluid retention. Rare but serious: venous thromboembolism, arterial thrombosis, jaundice and cholestatic liver injury, raised blood pressure, new migraine with aura. Refer to the current SPC.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • The date the last period started, and how pregnancy was excluded. • The reason the delay is wanted, and the dates it is needed for. • BMI, or the height and weight it was calculated from. • The answers to all eight Appendix 1 questions, including smoking status (and whether the patient stopped smoking less than a year ago) and the answer to the question about a journey of 4 hours or more, and the blood pressure measured today. • Where the patient is 16 or 17: the competence assessment and the safeguarding consideration, in full, not just the conclusion. • Number of days supplied and number of tablets supplied. • Any previous supply for period delay in the last 6 months. • That the patient was told norethisterone is not a contraceptive, and told to test for pregnancy if her period does not arrive. • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Advise the patient to return any unused tablets to a pharmacy.



Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	● Start 3 days before your period is due. Take one tablet three times a day, every day, until you stop. ● Your period will usually start 2 to 3 days after you take the last tablet. ● THIS IS NOT CONTRACEPTION. You can get pregnant while taking it. Use condoms or another method. ● If your period does not come within a few days of finishing, do a pregnancy test. ● If you think you could be pregnant while taking the tablets, stop them and do a pregnancy test. ● Move around and keep well hydrated, particularly on any journey where you are sitting for a long time. ● Some nausea, headache, breast tenderness, mood change or spotting is common and settles when you stop.
Follow-up, and when to get help urgently	Stop the tablets and get medical help the same day if you develop any of these: ● Pain, swelling, redness or warmth in one leg, particularly the calf. ● Sudden chest pain, sudden breathlessness, or coughing up blood. ● A severe or unusual headache, a new migraine, or any change in your vision or hearing. ● Yellowing of the skin or the whites of the eyes. ● Weakness or numbness down one side, or difficulty speaking. Call 999. For anything else, contact the pharmacy or your GP. If your period has not arrived within a week of finishing the course, do a pregnancy test and see your GP.

Appendix 1: The venous thromboembolism gate, as a checklist

Ask all eight. Any single YES excludes. Record the answers, not just the outcome.

1. Have you ever had a blood clot, a DVT, a clot on the lung, a stroke, a mini-stroke or a heart attack?	• YES: exclude and refer.
2. Has anyone in your immediate family had a blood clot before the age of 45, or been told they have a clotting disorder?	• YES: exclude and refer.
3. Do you smoke at all, or did you stop smoking less than a year ago?	• YES: exclude and refer. • Any amount, any age. Stopping less than a year ago excludes as well.
4. Height and weight, for BMI.	• BMI 30 or above: exclude and refer. • Measure or ask. Do not estimate.
5. Will you be on a flight, coach, train or car journey of 4 hours or more during the course, or in the 2 weeks after it?	• YES: exclude and refer. • This will exclude many holiday requests. That is the intended effect. Give the alternatives in Appendix 2.
6. Have you had an operation under general anaesthetic in the last 6 weeks, or do you have one planned during the course or within 2 weeks of finishing it?	• YES: exclude and refer.
7. Are you going to be immobile for any reason, for example a leg in plaster or bed rest?	• YES: exclude and refer.
8. Do you have cancer now, or have you been treated for cancer in the last 12 months?	• YES: exclude and refer.

Appendix 2: What to offer a woman who is excluded

An exclusion is not the end of the conversation. Every excluded woman should leave with something.

- If she takes a monophasic combined oral contraceptive: she can run packs back to back and skip the pill-free interval, for up to 3 consecutive packs. This is safer than norethisterone, needs no extra supply, and she can do it herself. Explain how.
- If she takes an everyday combined preparation: skip the placebo tablets and start the active tablets of the next pack.
- If she takes a progestogen-only pill: this cannot reliably delay a period. Do not advise doubling up. Refer.
- If she has an implant or injection: these often cause irregular bleeding or no bleeding, and have no role in actively delaying a period.



- If she is excluded on venous thromboembolism grounds: her GP can assess her individually and may still decide to prescribe. Say so, so that she does not hear a flat no.
- Practical measures for the event itself: menstrual cups, period underwear, and tranexamic acid or an NSAID for heavy or painful bleeding, subject to the usual checks.

Appendix 3: Key references

- NICE Clinical Knowledge Summary: Contraception, delaying or stopping periods.
- Faculty of Sexual and Reproductive Healthcare guidance on combined hormonal contraception, for the extended-use option in Appendix 2.
- Summary of Product Characteristics for norethisterone 5mg tablets, current version.
- British National Formulary.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, <https://www.nice.org.uk/guidance/mpg2>

Authorisation

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Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group	Name:



signatory

Job title and organisation:

Signature:

Date:

Change history to version 004

Later versions are recorded in the Version and change record at the end of this document.

Version	v004
Date	8 September 2026
Changes	• The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • Full clinical review and reissue. Re-read the document before working to it; the numbers have changed. • Maximum treatment period is 14 days. The previous document said up to 14 days in its summary and up to 17, extendable to 28, in its operative box. • Maximum quantity is 42 tablets, being 14 days at 5mg three times daily. The previous maximum was 90 tablets, which is 30 days. • Venous thromboembolism risk factors are now exclusions rather than cautions, set out as an eight question checklist in Appendix 1: personal or family history, current smoking of any amount at any age, BMI 30 or above, a seated journey of 4 hours or more during or within 2 weeks of the course, surgery within 6 weeks, immobility, and active or recently treated cancer. • A seated journey of 4 hours or more is an exclusion. This will exclude many holiday requests, which is the intended effect, and Appendix 2 sets out what to offer instead. • Age 16 and over is now formally authorised in a version signed by both signatories, with a required and recorded competence assessment, a safeguarding and coercion consideration, and a named referral route for every patient under 18. • Pregnancy exclusion made operable: the two questions to ask, when a pregnancy test is required, and the requirement to tell every patient to test if her period does not arrive. • Enzyme inducing medicines, ciclosporin and lamotrigine monotherapy moved from a general interactions statement into named exclusions. • Migraine with aura, uncontrolled hypertension or a blood pressure of 140/90 or above measured today, and diabetes with vascular complications added as exclusions. • Repeat supply limited to twice in any 6 months, and total treatment to 30 days in any 6 months. • Follow-up advice rewritten. The previous document carried template treatment-PGD wording telling patients to seek advice if symptoms "do not improve in 3-4 weeks", which is meaningless here. Replaced with named clot, liver and neurological symptoms requiring same-day help. • Alternatives for excluded women added as Appendix 2, including running monophasic combined pill packs back to back. • Broken NICE reference corrected from mg2 to mpg2.

Previous versions

001

Development and issue of new PGD. Norethisterone 5mg, women aged 18 and over. An amendment lowering the age criterion to 16 was recorded in the change history in July 2026 but was not reflected in any signed version; that change is properly authorised for the first time in v002.



Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

[illegible]



RISK FACTORS THAT DO NOT EXCLUDE ON THEIR OWN

The one UKMEC 2025 category 2 factor that this PGD does not exclude. On its own it does not exclude: supply, and counsel on the precautions below.

AGED 40 OR OVER. UKMEC 2.

SMOKING (any amount, any age), BMI 30 OR OVER, and a SEATED JOURNEY OF 4 HOURS OR MORE are EXCLUSIONS in this PGD (see the exclusion criteria and Appendix 1). They are not risk factors to be counted.

Record the risk factor where present, not only the conclusion.

PRECAUTIONS TO GIVE WHERE ANY JOURNEY IS INVOLVED (a seated journey of 4 hours or more excludes): move around at least hourly, keep well hydrated, avoid alcohol and sedatives on the journey, and consider graduated compression stockings. Seek urgent help for a painful swollen calf, sudden breathlessness or chest pain, during the course or in the weeks after it.



A NOTE ON USING UKMEC FOR A NON-CONTRACEPTIVE INDICATION

UKMEC states in terms that its recommendations are for CONTRACEPTIVE purposes only, and that where a method is used for a non-contraceptive indication the risk and benefit profile and eligibility criteria may differ. Period delay is not contraception.

UKMEC is therefore used here BY ANALOGY, not as directly governing criteria. The analogy is justified by the Primolut N SPC, which states that norethisterone is partly metabolised to ethinylestradiol, giving roughly 4 to 6 micrograms of ethinylestradiol per 1 mg of norethisterone, and that because of this the general warnings associated with combined oral contraceptives should also be considered.

At 5 mg three times daily that conversion implies an ethinylestradiol equivalent above that of a standard 30 to 35 microgram combined pill. That calculation is derived from the SPC figures, not stated in the SPC itself, and is recorded as derived so that nobody cites it as a licensed statement.

The current edition is UKMEC 2025, published December 2025 by the College of Sexual and Reproductive Healthcare, which was renamed from the Faculty of Sexual and Reproductive Healthcare in August 2025. Earlier versions of this PGD would have cited UKMEC 2016, which is superseded.

**BRAND MATTERS IN THIS PGD**

The three norethisterone 5mg products licensed in the UK do NOT carry the same safety information, and the differences bear directly on this service.

PRIMOLUT N (Bayer) is the only one that states the metabolism to ethinylestradiol, the combined-oral-contraceptive warnings that follow from it, smoking and age as thromboembolic risk factors, arterial thromboembolic risk, a maximum duration of 10 to 14 days for postponement, and depression aggravated as an adverse reaction.

UTOVLAN (Pfizer) carries the depression warning in its special warnings but not the ethinylestradiol conversion or the duration limit.

NORETHISTERONE 5mg TABLETS (Wockhardt) carries neither the depression warning nor the ethinylestradiol conversion, and states no maximum duration.

WHICHEVER BRAND IS SUPPLIED, THE WARNINGS IN THIS PGD APPLY. They are drawn from the fullest of the three. Do not read a shorter leaflet as permission to do less.



Version and change record

Version: v010

Issued: 14 September 2026

Supersedes: Version 009, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v009

Issued: 11 September 2026

Supersedes: Version 008, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Orphaned cover sentence (That gates nothing and is not auditable) removed; the sentence it referred to left in an earlier version.

Suspected pregnancy during the course is a stop-and-test instruction in the cautions and in the patient follow-up.

Records now require the answers to all eight Appendix 1 questions, the recent-quitter answer and the blood pressure measured today, and state that electronic records are acceptable.

Appendix 1 question 3 asks about stopping smoking less than a year ago, and question 6 carries the same time window as the exclusion (during the course or within 2 weeks of finishing).

Duplicated paragraph in the v004 change history entry removed.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v008

Issued: 11 September 2026

Supersedes: Version 007, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Exclusion list widened: hypertension of any grade (treated or not) and hypertension in pregnancy, atrial fibrillation and valvular or congenital heart disease, SLE and antiphospholipid syndrome, BRCA carrier status and breast cancer, and dyslipidaemia with another risk factor. The claim that the list 'mirrors UKMEC 3 and 4' is removed: the list is wider than UKMEC and says so.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v005

Issued: 9 September 2026

Supersedes: the previous signed version of this document, and any correction notice attached to it.

Change: An appendix heading carried "added at version 004" and a paragraph headed "why this changed" explained what versions 002 and 003 had got wrong, in the body of the document. Both removed. The rationale is in the change history for version 004 and remains there. No clinical change.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v006



Issued: 10 September 2026

Supersedes: Version 005, 9 September 2026

Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.

Version: v007

Issued: 10 September 2026

Supersedes: Version 006, 10 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

One rule for each venous thromboembolism risk factor, stated identically in the exclusion criteria, Appendix 1 and the risk factor page. Version 006 gave smoking three rules (exclude at 35 or over; exclude at any age; count as a risk factor under 35), BMI three thresholds (35, 30, and 30 to 34.9 as a counted factor) and long travel two (an exclusion on the cover and in Appendix 1; a counted factor on the risk factor page). The stricter rule is kept in each case, matching Appendix 1: any current smoker excludes, BMI 30 or over excludes, a seated journey of 4 hours or more excludes. The Medical Director confirmed the travel exclusion stands.

The risk factor page now lists the one category 2 factor this PGD does not exclude (aged 40 or over) and says that smoking, BMI and travel are exclusions rather than counted factors.



Signed on behalf of Get Real Health

Version v010, 14 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		14 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		14 September 2026

Both authorising signatories reviewed this version together on 14 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.